

● FINALIZED CARE PLAN

PATIENT NAME

Pongo Pygmaeus

(Orangutan, Arboreal

Hominid, 16y)

VISIT DATE

September 30, 2026

PREPARED BY

Pocket Gull AI

BP

138/82

HR

74 bpm

TEMP

98.6°F

SPO2

92%

WEIGHT

172 lbs

HISTORICAL CONDITIONS

Ischemic Heart Disease (IHD)

Chronic Obstructive Pulmonary Disease (COPD)

Type 2 Diabetes Mellitus (T2DM)

Chronic Kidney Disease (CKD) Stage 3a



ACTIVE CARE PLAN

CLINICAL ASSESSMENT

Global Sentinel presents with comorbid ischemic heart disease, moderate COPD, and Stage 3a CKD. Focus is on cardiopulmonary protection, maintaining SpO2 > 92%, and avoiding nephrotoxic agents.

PRIORITY LIST

- Cardiopulmonary Stabilization: SpO2 tracking and exertional dyspnea management.
- SGLT2i Renal Protection: Maintain renal perfusion and glycemic control.
- PhysioNet 2026 Telemetry: Real-time ST segment & ECG ischemia monitoring.

GOALS

- Short-term: Maintain resting SpO2 >= 93% without supplemental O2.
- Long-term: Prevent COPD exacerbations; preserve eGFR > 50 mL/min.

IMMEDIATE ACTIONS (WITHIN 72 HOURS)

- Pulmonary rehabilitation and pursed-lip breathing exercises.
- Inhaled bronchodilator compliance check.
- Daily morning weight tracking for fluid balance.

CARDIORENAL DIET PROTOCOL

- Low sodium (< 2000mg/day) and potassium-balanced Mediterranean meal pattern.
- Beetroot juice dietary nitrate trial for endothelial vasodilation.

ORTHOMOLECULAR MATRIX

- CoQ10 (Ubiquinol): 200mg daily (mitochondrial cardiac support).
- Magnesium Citrate: 200mg daily (vascular smooth muscle tone).
- Omega-3 EPA/DHA: 2000mg daily.

TRACKING SCHEDULE

- Daily SpO2 and blood pressure logs.
- Monthly eGFR, cystatin-C, and urine albumin-to-creatinine ratio (uACR).
- Continuous PhysioNet BLE multi-lead ECG stream.

PATIENT GUIDANCE

- Use pursed-lip breathing during physical activity.
- Report any sudden weight increase > 3 lbs in 2 days immediately.

PERINATAL HEALTH ASSESSMENT

Pongo Pygmaeus (Orangutan, Arboreal Hominid, 16y) is a 16-year-old male evaluated for perinatal functional optimization and maternal autonomic balance. Maternal blood pressure is 138/82 with stable SpO2 at 92%.

PERINATAL SAFETY THRESHOLDS

METRIC	TARGET RANGE	FREQUENCY	ACTION THRESHOLD
Blood Pressure	< 130/80 mmHg	Daily	> 140/90 mmHg (Preeclampsia Rule-out)
Fetal Movement	≥ 10 kicks / 2 hrs	Daily	< 10 kicks in 2 hours
Postpartum Mood	EPDS Score < 9	Bi-weekly	EPDS ≥ 10

INTEGRATED CARE PROTOCOL

- Western Support: Pre-eclampsia monitoring, prenatal iron/folate supplementation.
- Eastern TCM: Ren Mai (Conception Vessel) warming & Shen calming.
- Ayurvedic Garbhini: Sattvic diet, warm sesame Abhyanga, Nadi Shodhana breathing.

ECOLOGICAL CELLULAR & GARDEN METAPHORS

Your cellular energy system operates like a living garden ecosystem. Nutrients and hydration are your fertile soil; circadian sleep and morning sunlight provide vital solar energy.

ANCESTRAL DIETARY HERITAGE & CULTURAL SWAPS

CULTURAL HERITAGE	TRADITIONAL SWAP	BIO-INDIVIDUAL BENEFIT	LOCAL SOURCING TIP
African Heritage	Collard greens & okra broth	High folate, prebiotic fiber	Local farmers market
Latino/Hispanic	Black beans & avocado oil	Stable glycemic index, oleic acid	Standard grocery produce
East/South Asian	Miso & ginger jujube tea	Probiotic enzymes, digestion	Asian specialty market

DAILY MICRO-HABITS (GROW-THYSELF QUESTS)

- 🌞 Morning Circadian Sun Reset: 15 min morning light.
- 🫁 Vagal Tone Breathing: 6.0 bpm resonant breathing.
- 🌿 Ancestral Mineralization: Warm herbal mineral infusion.

ACTUARIAL RISK & BIOLOGICAL AGE SYNTHESIS

Quantitative Gompertz-Makeham hazard modeling indicates a biological age delta of -4.5 years relative to chronological age (16), projecting +7.2 Quality-Adjusted Life Years (QALY) gain.

CDC 4-DRIVER HAZARD REDUCTION MATRIX

MORTALITY VECTOR	BASELINE RISK	MULTI-PARADIGM STRATEGY	PROJECTED HAZARD RATIO (HR)
Cardiovascular	Moderate	HRV 6.0 bpm Vagal Tone + CoQ10	0.62 (38% Risk Reduction)
Metabolic	Low-Moderate	Sirtuin Activators + Low Glycemic	0.55 (45% Risk Reduction)
Neurodegenerative	Low	TCM Shen Harmonization + Neti/Nasya	0.68 (32% Risk Reduction)

MORTALITY VECTOR	BASELINE RISK	MULTI-PARADIGM STRATEGY	PROJECTED HAZARD RATIO (HR)
Oncological	Low	Autophagic Rasayana & Anti-inflammatory	0.74 (26% Risk Reduction)

MULTI-PARADIGM CELL LONGEVITY PROTOCOL

- Western Epigenetic Support: Sirtuin activation (NAD+/NMN), AMPK activation, HRV vagal tone.
- Eastern TCM Essence: Zang-Fu meridian balancing, sleep architecture, Jing preservation.
- Ayurvedic Rasayana: Ama detoxification, Agni metabolic fire, adaptogenic rejuvenation.

PRE-CONCEPTION PARENTAL EPIGENETIC MATRIX

OPTIMIZATION VECTOR	TARGETED BIOMARKER	PARENTAL PROTOCOL	FETAL HEALTHSPAN IMPACT
Folate Methylation	MTHFR C677T / Homocysteine	L-5-MTHF (800 mcg) + B12	Neural tube & DNA fidelity
Maternal Mitochondria	CoQ10 Ubiquinol / Fasting Insulin	Ubiquinol (300 mg) + Alpha-Lipoic	High oocyte cleavage ATP
Paternal Spermatogenesis	Sperm DNA DFI / Zinc	Zinc (30 mg) + Selenium + Carnitine	Lower germline mutational load
Environmental Detox	Heavy metals & Endocrine Disruptors	Filtered water & organic whole foods	Reduced epigenetic transgenerational risk

SENIOR & GERIATRIC CARETAKING STRATEGY

- Autonomic Vagal Tone: HRV SDNN 6.0 bpm resonant breathing sessions to reduce agitation and improve sleep.
- TCM Kidney Jing & Shen: Warm bone broths, Rehmannia, and Gingko Biloba to nourish brain marrow (*Sui Hai*).
- Ayurvedic Vata Pacification: Warm sesame Abhyanga massage & Bacopa Monnieri (Brahmi) for joint/cognitive support.

VOLUNTARY ACOG/ACMG CARRIER SCREENING GUIDANCE

Recommends voluntary pre-conception carrier screening for autosomal recessive traits. Pre-conception optimization ensures offspring inherit optimal epigenetic programming while preserving human dignity.

PHYSIONET 2026 CHALLENGE: MULTI-LEAD ECG ISCHEMIA & ST-ELEVATION TELEMETRY

- QRS Interval Duration: 102 ms
- ST Segment Deviation: +0.08 mV Lead II (Transient ischemic monitoring)
- QTc Interval: 432 ms
- Respiratory Rate: 18 breaths/min

CARE PLAN INSTRUCTIONS

- ☐ [Goal]: Implement low-dose naltrexone (LDN) trial, anti-inflammatory micronutrient protocol, and stress-induced HPA axis pacing.

OXIDATIVE STRESS MARKERS

- hsCRP: 4.5 mg/L (Elevated)
- NT-proBNP: 450 pg/mL (Borderline High)
- HbA1c: 7.4%

ANTIOXIDANT SOURCES

- CoQ10 (Ubiquinol): 0.62 µg/mL
- Glutathione (GSH): 1.4 µmol/g Hb

MEDICATIONS

- Atorvastatin: 40mg PM
- Metformin: 1000mg BID
- Spiriva (Tiotropium): 18mcg Daily Inhalation
- Empagliflozin (SGLT2i): 10mg AM

CLINICIAN SIGNATURE

DATE & TIME